Nursing Care of a Family in Crisis: Maltreatment and Violence in the Family

CHAPTER 55

Objectives

- Assess a family that has experienced physical or emotional child maltreatment or intimate partner violence
- Formulate nursing diagnoses related to a family in which child maltreatment or intimate partner violence is present.
- Develop expected outcomes for a family that has experienced child maltreatment or intimate partner violence to help them manage seamless transitions across differing healthcare settings
- Discuss the types of child maltreatment or intimate partner violence seen in families and the theories explaining their occurrence
- Implement nursing care for a family in which child maltreatment or intimate partner violence has occurred, such as ways to role model developmentally age-appropriate parenting
- Evaluate expected outcomes for effectiveness and achievement of care
- Integrate knowledge of family child maltreatment or intimate partner violence with the interplay of the nursing process, the six competencies of QSEN, and family nursing to promote quality maternal and child health nursing care

- A 3-year-old child presents in an emergency department. The parent tells you the child fell off a swing in the backyard. Upon exam and radiology reports, the child has a broken forearm, a broken rib, and multiple bruises on the chest and back. You notice in the electronic record that the child was seen in the same emergency room a month ago for a burn to the hand. When you mention to the parent that the child's injuries seem extensive for a simple fall, the parent says, "They are clumsy at this age."
- Previous chapters described the normal growth and development of children and care of children with disorders of specific body systems. This chapter adds information about the effect on children when child maltreatment or intimate partner violence occurs in a family. Such information can build a base for care and for putting into place ways to prevent further maltreatment or violence
- You suspect the child is a victim of child maltreatment. What additional questions would you want to ask the parent to assist with exploring this possibility? What should you do if you feel certain the child has been maltreated?

Maltreatment overview

- Child maltreatment is associated with stress and has been linked with the inability of a family to handle external and internal stressors. Accordingly, maltreatment or violence in a family is rarely an isolated event but rather an indication of many other factors that require intervention
- Maltreatment, formerly termed abuse, is defined as the “willful injury by one person of another” and takes many forms: physical or emotional maltreatment, neglect or sexual maltreatment, intimate partner violence, and maltreatment or violence of the elderly
- Maternity, child health, and family care nurses need to be especially observant for signs of possible maltreatment or violence in a family because these increase during pregnancy and possibly during the care of an ill child.
- In all instances, the victim’s safety is paramount, ensuring safety with sensitivity to the importance of maintaining and improving overall family functioning.
- Child maltreatment has long-term consequences; many children from families where there is child maltreatment become maltreating parents themselves

Assessment

- Nurses are often the first individuals to identify symptoms of possible child maltreatment or intimate partner violence in a family
- This is because nurses often obtain the initial health history or see a child or female undressed at a healthcare visit and recognize significant bruising; they may be the person to whom a pregnant person or child confides the problem
- If maltreatment or violence in any form is suspected, it is essential to get as complete details as possible
- If child maltreatment is suspected, talk with the parents first, without the child, and then interview the child separately to help uncover any inconsistencies in the parents' explanations
- Remember, however, you are not investigating the concern—you are doing an initial screening to assess the need for referral and reporting.
- Your agency's patient protective services department will do the actual investigation

Nursing diagnosis

- Nursing diagnoses associated with child maltreatment or family violence should address both the physical and the emotional results of the concern.
- Some examples are:
  - Pain related to burn on hand from documented child maltreatment Injury risk related to previous intimate partner violence
  - Other-directed violence risk related to admitted poor self-control
  - Altered parenting related to high level of stress Impaired family coping as manifested by child maltreatment related to alcohol use by parent or caregiver
  - Disturbed self-esteem related to stalking and sexual maltreatment

Outcome identification and planning

- Planning must center first on ensuring the safety of the maltreated family member and minimizing the effects of trauma
- Second comes reporting the discovery to authorities as nurses are mandated by law to report child maltreatment
- Long-term planning includes helping a maltreated family member find safe refuge and reestablishing self-esteem through a self-help or advocacy program
- Teaching empowerment, or the ability to take charge of one's life, is particularly important for older children and adults in families where there is maltreatment or violence
- In addition, the person who maltreated a child or an adult needs a program of therapy to help prevent future offenses

Implementation

- The most important intervention related to family child maltreatment or intimate partner violence is prevention
- Nurses can do much in all settings to be particularly observant for families who seem to be at risk for maltreatment or violence and role model optimal ways to handle family stress
- Further education is an intervention that can help parents who may not recognize their children's needs or normal child behavior to learn healthy patterns of childrearing

Outcome evaluation

- Expected outcomes should focus on specific examples of improved family interaction, such as the following:
  - The parent(s) hold baby in a caring manner and maintain good eye contact.
  - The parent(s) attend full series of counseling sessions on improving parenting skills.
  - The parent states they have the Crisis Center telephone number on their cell phone and will call for help if they feel at risk for harm.
  - The adolescent exhibits signs of positive self-image following an intimate partner violence episode. The parent attends monthly meetings of Parents Anonymous

Health Promotion and Risk Management

- Prevention must be the goal of healthcare providers to reduce the incidence of child maltreatment. Because many adults who maltreat children were maltreated themselves, stopping the cycle of abuse will help prevent maltreatment in future generations
- (Box 55.2). Identify children who may be viewed as special in some way by parents, such as those who were separated from the family at birth, were premature, or are physically or cognitively challenged, because these children are at high risk for being victims of maltreatment.

- Identifying parents who may be at risk for maltreating their child is an initial step in prevention
- Encourage parents to take an infant care course
- Another nursing responsibility aimed at preventing child maltreatment is helping young parents learn about normal growth and development of children and how to better respond to the child's behavior
- Parents may be identified as being at risk for child maltreatment during health maintenance visits Box 55.3

Child Maltreatment

- Parenting is not an easy task and good parenting is not automatic or instinctive, children in every community are at risk
- Not only places child at risk for immediate harm but can also have long-term effects
- When children reach adulthood and begin parenting, they tend to raise their children the same way as they were raised.
- Parents who themselves received little love or were maltreated as children may never form a basic sense of trust and so may not be able to form a sense of intimacy or the ability to care for others as they mature

Maltreatment

- Differentiate between normal growth and development and abuse, and other disease processes and abuse
  - Toddlers have ecchymosis spots in their legs
  - Child with leukemia has lots of bruising all over body
  - Child with osteogenesis imperfecta
- Red Flags – need further assessment!
  - Spiral fracture-Bone break, someone has to worstly squeeze hands.
  - Skull fracture
  - Any fracture in an infant who is nonmobile is suspicious
  - Always assess for injuries that do not match a child's developmental level or abilities or injuries that do not match the explanation of how the injury occurred

Special Parent – Parent Who Maltreats

- Risk factors for parents who maltreat children include substance abuse, mental health issues, young age, low education and income, single parent status, and having multiple dependent children
- Maltreated as a child
- Not educated on normal growth and development and have unrealistic expectations of child
- Socially isolated with no support
- strongly associated with parental use of drugs such as alcohol, substances that remove inhibitions and self-control

Special Child – Children Who Are Maltreated

- Children who are maltreated tend to be viewed by parents as somehow “different.”
- They may be more or less intelligent than other children in the family; they may have been unplanned; or they may not live up to their parents’ expectations in some way
- Children younger than 4 years are at greater risk for child abuse than older children as are those who require an increased caregiver burden such as children with intellectual disabilities, mental health issues, and chronic illness. These children cannot verbalize that they are being hurt.
- To prevent maltreatment, a child may assume a role reversal with the parent or become the comforting, consoling individual. Child being supporting parents like they reverse role.
- It is important to assess who is comforting whom when a childhood injury occurs.

Special Circumstances: Stress

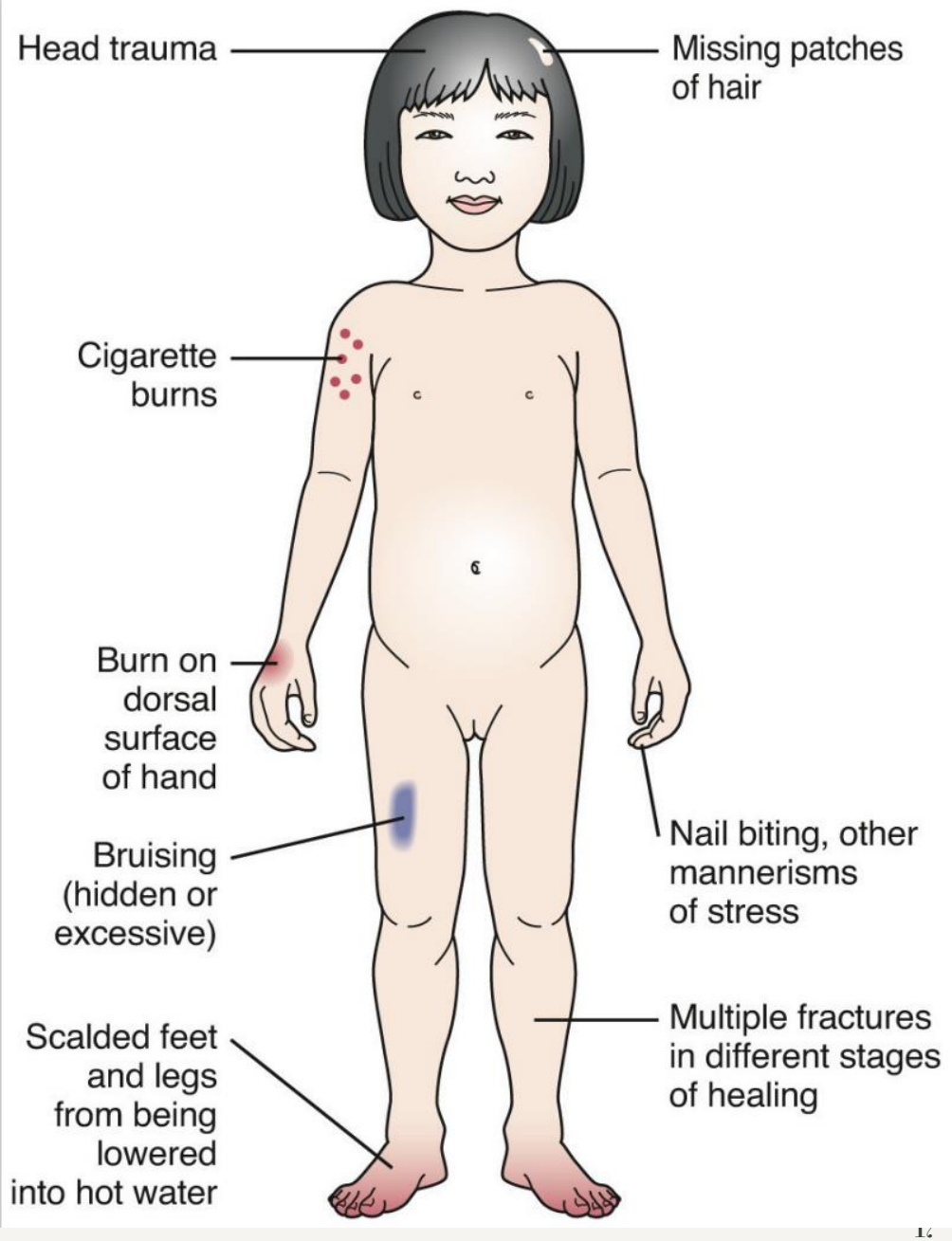
- The third factor in child maltreatment is stress
- It might be something as common as a broken appliance, a minor illness in a family member, a rainstorm that cancels a picnic, or something more impactful like a lost job and/or the inability to pay for necessities and housing
- Child maltreatment crosses all socioeconomic levels because stress is a part of life for everyone at one time or another
- Stress generally has a greater impact on individuals who do not have sources of support or who have poor coping abilities.
- Families whose internal support system is not strong or who have not formed outside support systems are likely to have a higher incidence of maltreatment
- Individuals who are resilient and adapt well when faced with adversity experience less stress

Reporting Suspected Child Maltreatment

- State laws typically identify two levels of responsibility for reporting child maltreatment:
  - Mandatory reporters and permissive reporters.
  - Permissive reporters are encouraged to report suspected child abuse but are not required by law.
  - Mandatory reporters are professionals who are mandated by law to report child maltreatment
- Nurses are mandatory reporters!!!!
- A second provision in state laws is protection from having a lawsuit brought against a healthcare provider for reporting suspected maltreatment "in good faith" that is then proved false

Physical Maltreatment

- Physical maltreatment is the action of a caregiver that causes intentional physical injury to a child.
- The child may exhibit burns, bruises, and fractures know this picture signs.

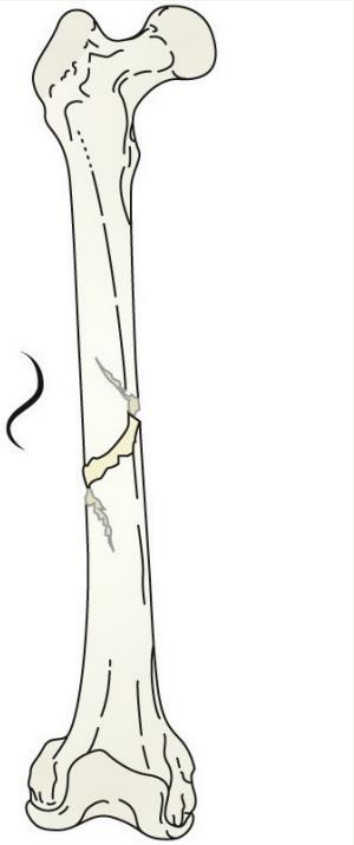


Assessment

- When obtaining a history, ask caregivers to account for any injury to a child's body
- injury is out of proportion to the history given by the parent or caregiver
- parents may give conflicting stories (one parent says the child fell, but the other parent says the child broke the arm while throwing a ball)
- may give no reason for the injury
- Ask children about the injury as well as the parents because if they did not hear the parent's explanation of the injury, they may say something that differs from the parent's explanation
- To remain objective, always assume the parents have done the best they could under the circumstances in which they found themselves. The fact that they have brought the child for care means they are seeking help; it may be their way of saying, "Help me; I don't want this to happen again." In many instances, child maltreatment is not an isolated phenomenon as a parent is also a victim and needs help and protection, as does the child

Physical Examination

- When children are examined at a well-child visit or because of illness, be certain they are fully undressed (including removing all bandages and adhesive bandage) so their entire body can be observed. Plot height and weight on a standard growth chart at every visit. Delays in growth may suggest neglect
- Children who are beaten with electrical cords, belts, or clotheslines have peculiar circular and linear lesions
- Children who are beaten with a belt buckle may have additional curved lacerations from the imprint of the buckle
- Burns
- Cigarette Burns
- Other common findings include multiple fractures in different stages of healing
- Box 55.5



Abusive Head Trauma – difficult to prove.

Abusive head trauma, previously known as shaken baby syndrome, involves repetitive, violent shaking of a small infant by the arms or shoulders, causing a whiplash injury to the neck, edema to the brain stem, and distinct retinal hemorrhages

Symptoms: edema to the brain stem, possibly subdural hemorrhage, and distinctive hemorrhages to the retinas

Can be difficult to prove

Use of CT and MRI to help detect with internal images

Physical Neglect

- More subtle form of maltreatment but still just as damaging to child's welfare
- Reflects 60% of maltreatment cases in US
- Two most common are not providing for child's basic needs and leaving them unattended
- Failing to bring a child for medical attention or failing to seek early medical care for an infection are other signs of neglect. Not requiring a child to attend school, deliberately keeping a child out of school without setting up a home school program, or allowing a child to go unsupervised after school may also be interpreted as neglect
- Many parents need anticipatory guidance from healthcare personnel to better understand their child’s needs and developmentally appropriate parenting skills

Psychological Maltreatment

- Psychological maltreatment includes constant belittling or threatening, rejecting, isolating, or exploiting a child or is the absence of positive parenting
- This type of maltreatment is the most difficult form of maltreatment to detect because it may occur only in the home, and its effects, although severe, may be subtle
- Some indicators of emotional abuse include
  - Developmental delays
  - Bed-wetting without medical cause
  - Frequent psychosomatic complaints
  - Severe depression
  - Anxiety or aggression
  - Self-destructive behaviors such as self-harming
  - Overly compliant child

Medical Child Abuse

- Formally known as Munchausen syndrome by proxy, refers to a parent who repeatedly brings a child to a healthcare facility and exaggerates, invents, or induces symptoms of illness when, in fact, the child is well
- Two classic findings of the syndrome are usually present: First, the symptoms are not easily detected by physical examination but only by history; and, second, the symptoms are present only when the person initiating the symptoms is providing care
- The parent usually has some degree of medical or childcare knowledge obtained through formal education, reading, or internet browsing
- In the hospital, the parent tends to stay with the child constantly, offering to give the majority of care

Failure to Thrive

- In the hospital, the parent tends to stay with the child constantly, offering to give the majority of care
- Usually divided into two categories: one where severe loss of weight can be explained because of organic causes, such as cardiac disease, and a second that occurs because of a disturbance in the parent–child relationship, resulting in parental role insufficiency (a nonorganic cause)
- Failure to thrive begins with subtle signs that need to be identified and acted upon because the syndrome can lead to cognitive impairment in the child and even death if allowed to continue
- With advanced failure to thrive, the child's nutritional status may be so extremely poor that the infant is near acidosis from starvation

Failure to Thrive

- Lethargy with poor muscle tone, a loss of subcutaneous fat, or skin breakdown
- Lack of resistance to the examiner's manipulation, unlike the response of the average infant
- Rocking on all fours excessively, as if seeking stimulation
- Possibly a greater reluctance to reach for toys or initiate human contact than is demonstrated by the average infant; diminished or nonexistent crying
- Staring hungrily at people who approach them as if they are starved for human contact
- Little cuddling or conforming to being held
- Delays in sitting, pulling to a standing position, crawling, and walking because the child spends so much time alone
- Markedly delayed or absent speech because of the lack of interaction

Failure to Thrive Nursing Diagnosis

- Malnutrition related to inadequate intake secondary to emotional deprivation
  - Ensure adequate nutrition
  - Nurture the child
  - Support and encourage the parents
  - Help prevent poor parenting

Sexual Maltreatment

- Broadly defined as any sexual contact between a child and an adult
- Child sexual abuse is evident when the activity is intended to gratify or satisfy the needs of the other person
- It involves the coercion of dependent, developmentally immature children or adolescents in sexual activities that they do not fully comprehend, to which they are unable to give informed consent, or that violate the social taboos of society
- Problem of epidemic proportions
- Anyone can be an offender
- On physical exam, females may have tears or inflammation of the vagina or perineum; both sexes may have rectal tears or symptoms of a sexually transmitted disease – less than 5% will show physical signs
- Megan's Law

Types of Sexual Maltreatment

- Molestation vague term that includes “indecent liberties” such as oral–genital contact, genital fondling and viewing, or masturbation
- Pornography and Prostitution involves photographing or describing sexual acts by any medium involving children or distributing such material in person or by mail, fax, cell phone, or over the internet. Child prostitution involves arranging or participating in sexual acts with children
- Incest is sexual activity between family members. It often involves an older family member and a younger family member

Identifying Signs of Sexual Maltreatment

BOX 55.6

Nursing Care Planning Based on Family Teaching

IDENTIFYING SIGNS OF SEXUAL MALTREATMENT

Q. A parent asks you, "What signs should I look for if I suspect my child is being sexually maltreated?"

A. Here are some general indications that a child is being sexually maltreated:

- The child reports sexual activity with an adult.
- The child demonstrates an awareness of sex or a sexual vocabulary beyond age expectations.
- The child participates in sexual expression with dolls.
- A child younger than 15 years is pregnant.
- Examination of the genitalia reveals inflammation, tears, or anal fissures.
- A child younger than 15 years has a sexually transmitted infection.
- A child has a history of symptoms of increased anxiety such as a sleep disturbance, development of nervous tics, nail biting, or stuttering.
- There is a change in school performance, school phobia, or truancy.
- The child voices a fear of being left alone with a certain adult.
- A child reports vague abdominal pain or demonstrates acting-out behavior.

Therapeutic Management

- Parents need counseling when their child has been sexually maltreated so they can help the child work through feelings about the situation
- Often, the offender is a family member or a trusted outsider, and the relationship has been occurring for some time before it is reported
- Parents may feel guilty they allowed the family member to have access to the child or did not listen to a child's protestations that being alone with this family member was uncomfortable for the child
- If incest involves a parent, marriage, or relationship partner, it may be extremely difficult for parents or partners to continue to relate to each other effectively enough to help the child
- Review with parents and encourage parents to teach children simple rules to help them avoid sexual maltreatment

Rape

- Sexual activity such as intercourse or penetration of a body orifice by a penis or other object under actual or threatened force
- Rape trauma syndrome, a form of posttraumatic stress syndrome, and they usually occur in two stages: disorganization and reorganization
- Nurses serve as important members of such teams and may provide primary care after a rape. Any nurse should be able to offer emergency support because it might be a long time before a specifically designated staff member arrives, and such services are not available in every community
- Sexual assault nurse examiner (SANE nurse) should be contacted if available
- Quote the victim's exact words whenever possible.
- Describe the victim's physical appearance in unbiased detail, including the presence and location of injuries, such as bruises, lacerations, teeth marks, or abrasions, and the condition of clothing
- Be certain to ask whether the victim bathed or washed before coming for care because this can obscure evidence and obliterate the presence of DNA material

Goals of Crisis Intervention for Families of Victims of Rape

- Help the family be supportive of and reassuring to the victim to help initiate problem-solving techniques.
- Explain the possibility of future psychological and somatic symptoms that characterize a rape trauma syndrome and what the family can do to minimize these symptoms.
- Eliminate the family's sense of guilt for not protecting the victim by assuring them they could not have anticipated or prevented the rape.
- Educate the family about rape as a *violent crime*, not a sexually motivated act, to eliminate a focus on the victim's guilt or responsibility.
- Discourage violent, destructive, or irrational retribution toward the rapist (under the guise of being on the victim's behalf) by encouraging sharing of feelings of helplessness, sadness, hurt, and anger.
- Explain the possibility of sexually transmitted disease and pregnancy that may result from a rape, the preventive care necessary for the victim and spouse or sexual partner, and the follow-up care indicated.
- Refer the family for counseling if members' shared responses to the crisis interfere with their ability to cope adaptively.
- Identify how the family has handled crises in the past and encourage members to use adaptive coping mechanisms for this crisis. Encourage contact with persons identified as supportive to the family and offer to contact such persons.

Legal Considerations

- Nurses working in emergency departments may be asked to testify in court about a victim's appearance after the assault, although the documentation in the record is usually all that is necessary
- Most victims do not file charges

Wrap Up

- A triad of “special parent, special child, and special circumstance” is characteristic of the family in which child maltreatment occurs
- Children who comfort parents in emergency settings may just be sensitive children or may be demonstrating role reversal, a behavior characteristic of children who are maltreated
- Methods to prevent maltreatment in which nurses can actively participate include teaching about the expected growth and development of children, educating parents for parenting roles, and teaching empowerment or a sense that children and adults have control of their own lives.
- Sexual maltreatment of children can be prevented by educating children to recognize inappropriate touching and to empower the child to speak out if touched in private parts or forced to participate in activity that makes them feel uncomfortable. They should not be asked to keep inappropriate behavior a secret for fear of self or family harm.
- Maltreatment is a family problem, not an individual problem. Therapy must include all family members to be effective

Child Abuse Awareness Month – April

In 2024, there were 783 confirmed victims of child abuse in Lubbock County
1 death